

A wound that appears sealed

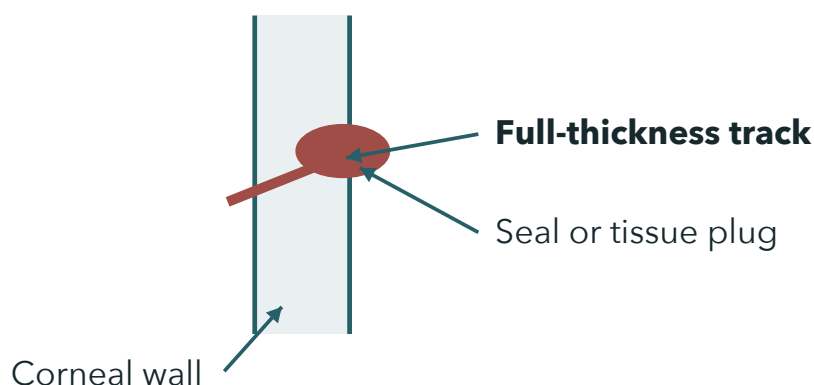
CLINICAL M3-M5 | TYPE B | A focused decision guide

Do not let a negative test close the case

Why leakage may be absent

Outside

Anterior chamber



No visible leakage does not establish an intact globe.

Original corneal section, not to scale. A wound may seal or be plugged despite full-thickness injury.

History determines remaining suspicion

- 1** High-speed projectile, sharp object or severe blunt force: assess injury mechanism and visual loss.
- 2** A negative Seidel result cannot exclude a self-sealed, plugged or posterior wound. Do not provoke leakage.
- 3** If suspicion persists, protect without pressure and obtain urgent specialist examination and appropriate CT.

Avoid pressure

Defer tonometry and contact ultrasound when an open globe is suspected. Do not remove a protruding object; prevent straining while definitive care is arranged.

Early and delayed consequences

Complication changes the response

Pattern after injury	Concern
Pain, hypopyon, worsening vision	Intraocular infection: urgent specialist treatment.
New flashes, curtain or vitreous blood	Retinal break or detachment: assess the posterior segment.
Bilateral granulomatous inflammation later	Consider sympathetic ophthalmia after penetrating trauma or surgery.

Bilateral inflammation after unilateral injury

- 1 Recognise that the fellow eye may be affected by an immune process.
- 2 Arrange urgent uveitis care and assessment for competing infectious causes.
- 3 Specialist anti-inflammatory and often steroid-sparing care aims to preserve both eyes.

A retrospective trap

Do not teach late eye removal as an automatic cure for established sympathetic ophthalmia. Management depends on timing, visual potential and specialist assessment.